Which treatment this guide covers NCCP 00220a covers adults with locally advanced or metastatic NSCLC with an activating EGFR mutation. This is continuous treatment for advanced disease, not a fixed adjuvant course.

NCCP 00220 · VERSION 5

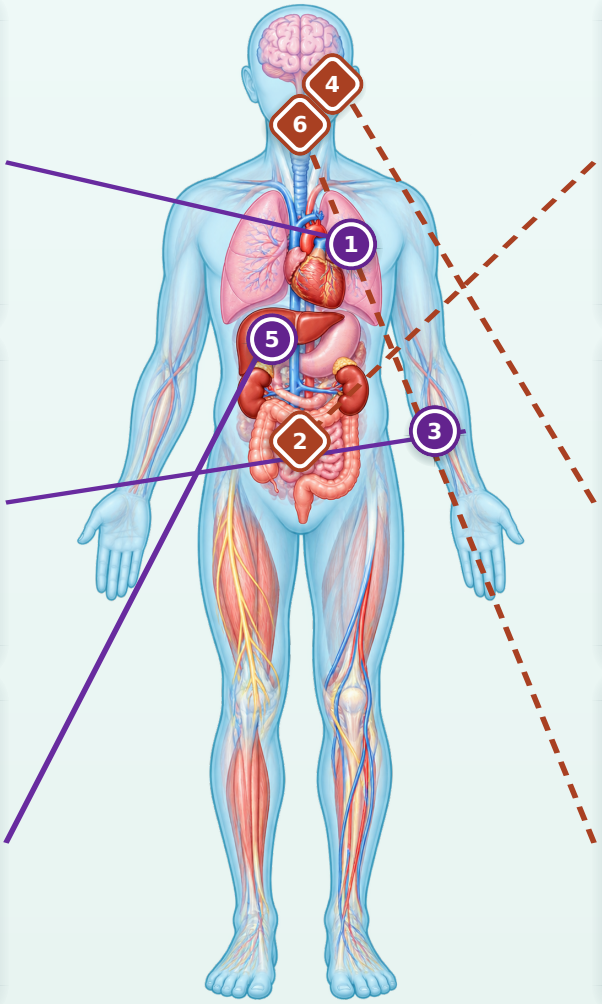
Gefitinib monotherapy

The usual dose is 250 mg by mouth once daily, continuously without a planned rest week. Use the dose on your prescription. Treatment continues while it helps and side effects remain acceptable; any change is agreed with your team.

Daily EGFR-targeted tablets · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



1 Lungs

gefitinib

New cough or breathlessness needs urgent assessment.

2 Bowel

gefitinib

Diarrhoea can cause dehydration; follow the symptom plan.

3 Skin and nails

gefitinib

Report rash, painful nails or peeling skin.

4 Eyes

gefitinib

Eye pain or blurred vision needs prompt review.

5 Liver

gefitinib

Blood tests check for liver injury.

6 Mouth

gefitinib

Report soreness or difficulty eating.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call urgently for new or worsening breathlessness, cough or fever; severe diarrhoea, inability to drink or much less urine; severe or persistent abdominal pain; blistering or peeling skin; or yellow skin/eyes. Eye pain, light sensitivity or blurred vision needs prompt assessment. Severe breathing difficulty, collapse or swelling of the tongue/throat needs emergency help. Use your unit’s temperature and 24-hour contact instructions. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

YOUR PRACTICAL TREATMENT PLAN

Know the next step.

Taking treatment

The usual dose is 250 mg by mouth once daily, continuously without a planned rest week. Use the dose on your prescription. Treatment continues while it helps and side effects remain acceptable; any change is agreed with your team.

Take once daily with or without food, at about the same time. If missed, take when remembered unless less than 12 hours remain before the next dose; then skip it. Never double a dose. If swallowing is difficult, pharmacy can teach dispersion in non-carbonated water without crushing; drink within 60 minutes and rinse the glass. Ask your team what to do after vomiting; do not take an extra tablet without advice.

Checks and follow-up

NCCP specifies blood count, kidney and liver tests throughout treatment as clinically indicated. The team sets your appointments and scan plan. Report symptoms promptly; extra checks may be needed for diarrhoea, dehydration or liver problems.

The whole-treatment map shows the checks for this exact treatment setting.

Support between reviews

Diarrhoea and fluids

Keep the prescribed anti-diarrhoea plan available from the first dose. Report loose stools early and keep drinking if able. Persistent or severe diarrhoea, weakness, inability to drink or much less urine requires urgent advice. The team decides whether to hold treatment and when it is safe to restart.

Skin, nails and mouth

Use the agreed skin, nail and mouth-care plan. Report rash, painful nails or mouth sores early. Emollients and sun protection may help; the team decides whether creams, antibiotics or other treatment are needed. Blistering or peeling skin needs urgent assessment.

Nausea medicines

Gefitinib has minimal-to-low oral emetic risk. Agree any as-needed anti-sickness medicine with the team. Routine infusion premedication is not required.

Medicine interactions

Give pharmacy all medicines and supplements. Strong CYP3A4 inducers can reduce exposure; inhibitors and some CYP2D6 interactions can increase risk. Medicines that keep stomach acid low may reduce absorption. Ask for an individual medicine plan; do not borrow timing from another EGFR medicine. Warfarin needs regular blood-clotting checks.

Pregnancy, fertility and breastfeeding

Avoid pregnancy during treatment and discuss effective contraception and future pregnancy plans with the team. NCCP excludes pregnancy and breastfeeding; do not breastfeed during treatment. The cited product information does not specify a universal post-treatment contraception interval, so ask for an individual plan.

Oncology daytime: Insert numbers here. **Out of hours:** Insert numbers here. Follow your own hospital’s urgent-care plan. Emergency: 112 / 999.

Your complete treatment reference

https://sactcheck.com/patient/00220/

Public information only. No personal information is encoded.